

Public Health Emergency Operations Center
“COUSP DRC”
MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°072/MVB_25/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (48)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Lubunga, Makiso-Kisangani and Mangobo
Reporting date	July 25, 2026
Publication date	July 26, 2026

INDICATORS KEYS — CURRENT TO JULY 25, 2026

CONFIRMED CASES — 5 PROVINCES 3,200 +125 new cases (Ituri 115 · N-Kivu 7 – Haut-Uele 3) 	CONFIRMED DEATHS · LEATHITIS 1,405 · (43.9%) +51 deaths (Ituri 42 · North Kivu 8 - Upper Kivu) Uele 1) 	SUSPECTED CASES OF THE DAY 340 Ituri 242 · N-Kivu 86 · S-Kivu 7 · Haut-Uele 3 · Tshopo 2 
PATIENTS IN ISOLATION 773 Ituri 602 · North Kivu 171 	CURED — CUMULATIVE 571 +15 recovered (Ituri 13 – North Kivu 2) 	NATIONAL CONTACT TRACKING 77.8% 12,010/ 15,440 views · Ituri 75.6% - Top-Uele 64.8% - N-Kivu 83.7% - Tshopo 90.9% 

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected. The total remains at 48 affected health zones.

140 in the five provinces, of which 47 remain in active transmission.

- **Coordination:** Supervision of response actions by pillar.

- **Case trends:** In the last 24 hours, 125 new confirmed cases, 51 confirmed deaths, and 15 recoveries.

were recorded:

- o **Ituri:** 115 new confirmed cases were notified, mainly in Rwampara (35), Bunia (23), Nizi (20), Nia-Nia (11), Lita (8), Mongbwalu (8), Damas (2), Komanda (2), Mambasa (2), Mangala (2) and Mahagi (1), 42 deaths among which 66.7% were community deaths.

North **Kivu:** 7 new confirmed cases were reported (Butembo 4, Katwa 2 and Beni 1) as well as 8 deaths, 50.0% of which were community transmission. The provincial total reaches 306 confirmed cases and 196 deaths, representing a case fatality rate of 64.1%.

- o **Haut-Uélé :** 3 new confirmed cases (1 case in Boma Mangbetu, 1 case in Isiro and 1 case in Wamba) and 1 death at the Ebola Treatment Center (ETC) have been reported. The provincial total has risen to 38 confirmed cases and 20 deaths, representing a case fatality rate of 52.6%, spread across 5 of the province's 13 health zones.

o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The cumulative totals remain at 5 cases, 4 deaths and 1 recovery in Tshopo and 3 cases, including 1 death and 2 recoveries in South Kivu, where the Miti-Murhesa health zone has gone 60 days and the Tshopo province 11 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 89.0% of cumulative cases and 92.0% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 48 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

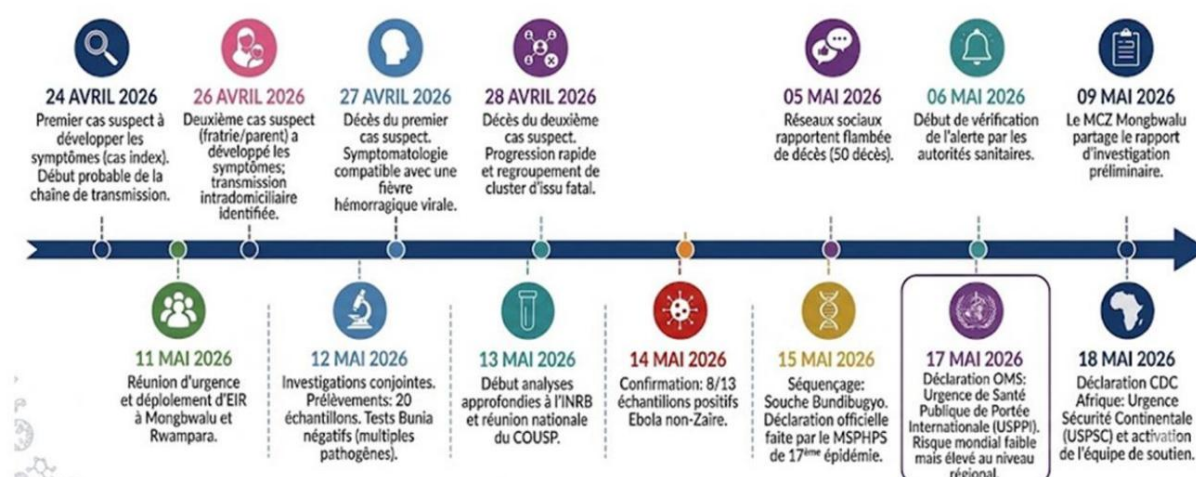


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,848	1184	41.6%	28 / 36 (77.8%)	115
North Kivu	306	196	64.1%	11/34 (32.4%)	7
Haut-Uélé	38	20	52.6%	5/13 (38.5%)	3
Tshopo	5	4	80.0%	3/23 (13.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,200	1,405	43.9%	48 out of 140	125

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 89.0% of confirmed cases (n = 2,848) and 84.3% of deaths (n = 1,184), resulting in a case fatality rate of 41.6%. The health zones of Bunia (762 cases), Rwampara (550), Mongbwalu (498 cases), Nizi (350), Nyankunde (114), and Lita (117 cases) remain the most affected, accounting for approximately 84.0% of reported cases at the provincial level and 74.7% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,848	1184	41.6%	115	28	14	42
Adja	11	1	9.1%	1	0	0	0
Aru	5	4	80.0%	0	0	3	3
Ariwara	7	1	14.3%				
Aungba	4	4	100.0%				
Bamboo	54	11	20.4%				
Boga	1	1	100.0%				
Bunia	762	215	28.2%	23	11	4	15
Damascus	16	8	50.0%	2	0	0	0
Drodro	6	4	66.7%				
Fataki	17	6	35.3%				
Gethy	1	0	0.0%				
Kambala	2	0	0.0%				
Kilo	23	11	47.8%				
Komanda	35	24	68.6%	2	2	0	2
Lita	117	66	56.4%	8	3	1	4
Logo	9	3	33.3%				
Lolwa	4	1	25.0%				
Mahagi	3	1	33.3%	1	0	0	0
Mambasa	9	4	44.4%	2	0	0	0
Mandima	13	9	69.2%				
Mangala	100	64	64.0%	2	1	0	1
Mongbwalu	498	237	47.6%	8	3	2	5
Nia Nia	89	46	51.7%	11	1	0	1
Nizi	350	166	47.4%	20	7	1	8
Nyankunde	114	31	27.2%	0	0	1	1
Rimba	8	4	50.0%				
Rwampara	550	238	43.3%	35	0	2	2
Tchomia	40	24	60.0%				
North Kivu	306	196	64.1%	7	4	4	8
Blessed	43	30	69.8%	1	1	0	1
Butembo	70	48	68.6%	4	3	2	5
Goma	1	0	0.0%				
Kalunguta	4	3	75.0%				
Katwa	138	90	65.2%	2	0	2	2
Kyondo	11	7	63.6%				
Mabalako	1	0	0.0%				
Masereka	4	0	0.0%				
Musienene	29	14	48.3%				
Oicha	4	3	75.0%				

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Vuhovi	1	1	100.0%				
South Kivu	3	1	33.3%	—			—
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	38	20	52.6%	3	0	1	1
Boma Mangbetu	7	4	57.1%	1	0	1	1
Isiro	12	7	58.3%	1	0	0	0
Pawa	7	5	71.4%				
Rungu	1	0	0.0%				
Wamba	11	4	36.4%	1	0	0	0
Tshopo	5	4	80.0%	—			—
Lubunga	1	0	0.0%				
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Total	3,200	1,405	43.9%	125	32	19	51

2.3 Mapping of confirmed cases

The spatial distribution confirms that transmission is mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwatu, Nizi, Nyankunde and Lita, with transmission continuing in already affected areas, including the Rungu health zone in Haut-Uélé.

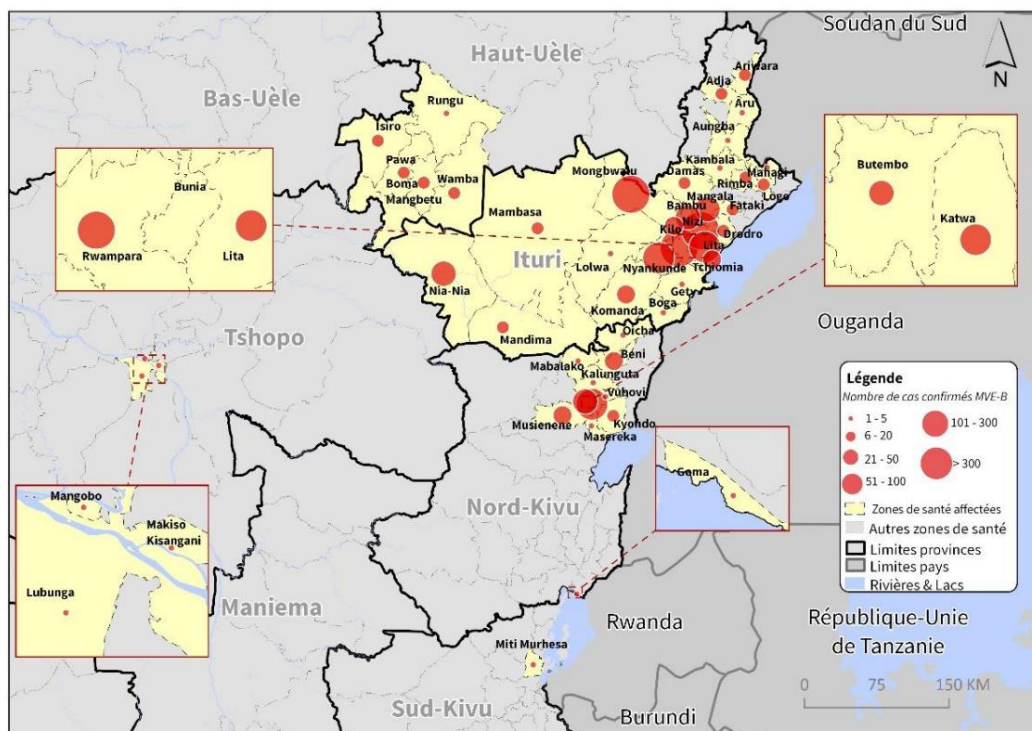
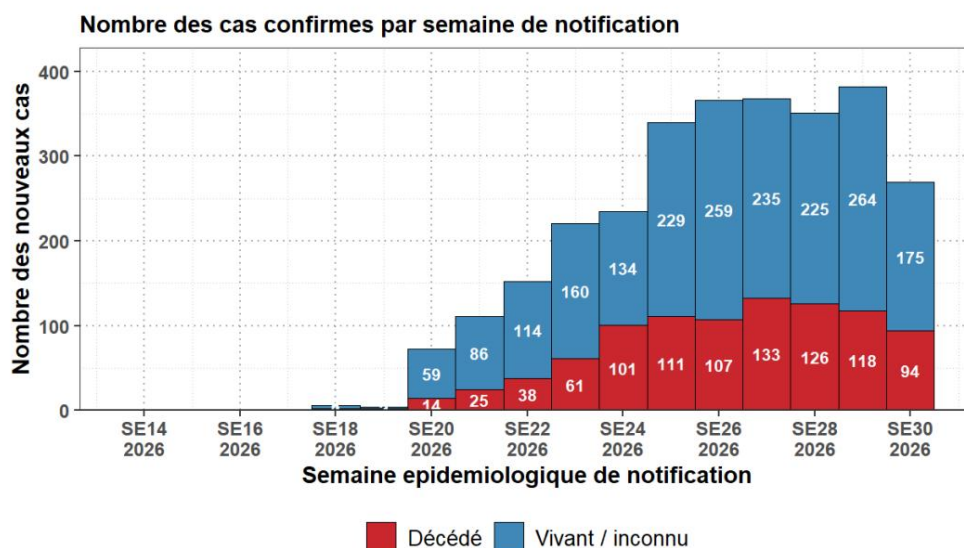


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

The continued transmission in the Rungu health zone in Haut-Uélé demonstrates the continued transmission in already affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

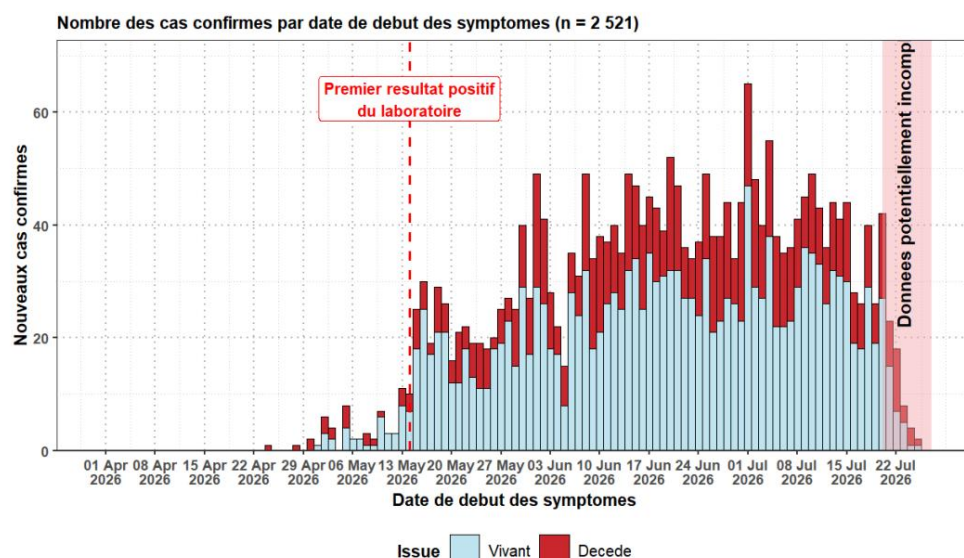
2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 878.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de début des symptômes renseignés, n = 2 521.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

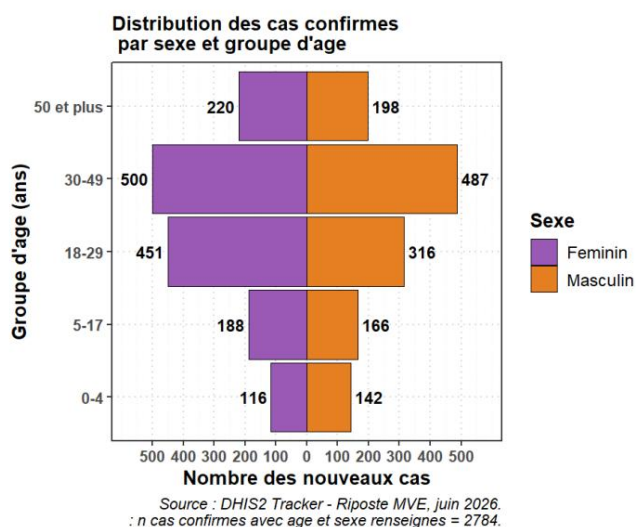
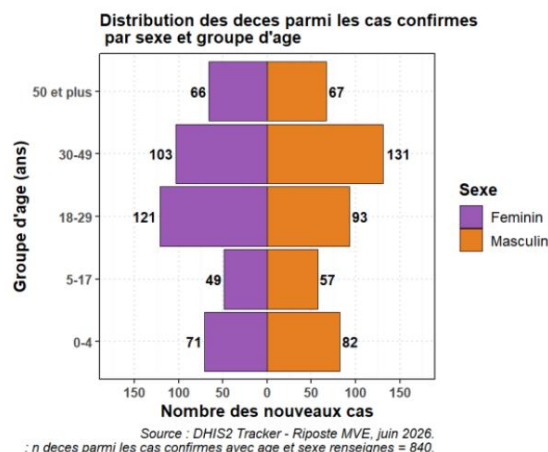


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	7	33	41	0	1	82
New alerts received	581	714	11	22	7	1,335
Total alerts for the day	588	747	52	22	8	1,417
Alerts investigated	482	707	6	4	7	1,206
Investigation rate (24 h)	81.9%	94.7%	11.5%	18.2%	87.5%	85.1%
Alerts confirmed — live	168	54	2	2	7	233
Alerts confirmed — deceased (comm.)	74	32	1	0	0	107
Total suspected cases for today	242	86	3	2	7	340

In total, 1,417 alerts (including 1,335 new ones) were received, with 1,206 investigated within 24 hours (85.1%) and 340 validated (233 active cases and 107 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation delays and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

Ÿ **Ituri** : 224 samples documented in the collection network, of which 115 were positive (87 living and 28 deceased), representing a positivity rate apparent of 51.3%.

Ÿ **North Kivu** : 135 samples received and analyzed, of which 7 were positive (4 living and 3 dead), representing a positivity rate of 5.2%.

Ÿ **Haut-Uélé** : 10 samples received and 8 analyzed, among which 2 were positive (live), i.e. a positivity rate of 25%.

Ÿ **Tshopo** : 5 samples received and analyzed. All of these samples came back negative after analysis.

- One (1) case in the Wamba health zone after harmonization of the database in the Haut-Uélé.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	10,901	8,245	75.6%
North Kivu*	4,307	3,606	83.7%
Tshopo	33	30	90.9%
Haut-Uélé	199	129	64.8%
South Kivu (end of monitoring)	0	0	N / A
Total	15,440	12,010	77.8%

* In Ituri and North Kivu, the monitoring rate was calculated only for health zones that had submitted reports. The health zones of Damas, Fataki, Kilo, Kambala, Logo, Mambasa, Nyankunde, Rimba, Masereka and Mutwanga did not have data available at the time of report consolidation.

Contact tracing remains insufficient at the national level (77.8%), below the operational threshold of 95%. Ituri only managed to follow up on 8,245 out of 10,901 contacts (75.6%). North Kivu achieved a performance of 3,606 out of 4,307 contacts (83.7%). Haut-Uélé only followed up with 129 out of 199 contacts (64.8%). Tshopo maintains a relatively high performance with a 90.9% follow-up rate. Follow-up is closed in South Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 9 alerts notified

Nine (9) alerts were intercepted at the PoE/PoC: 7 live alerts and 2 deceased individuals. Of the 7 live alerts, 6 were validated and all referred to the CT/CTE; 1 was invalidated after secondary screening. The 2 deceased individuals were swabbed.

North Kivu — 2 alerts notified

Two (2) lifeless bodies were intercepted at PoC Kasitu and referred for swab.

Tshopo — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

Haut-Uele — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	Tshopo	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16) 21.1% (4/19)		84.2% (80/95)
Number of people who have switched to PoE/PoC	98,423	53,748	3,765	155,936
% of travelers screened %	95.3%	96.9%	98.6%	96.0%
of travelers who washed their hands	92.9%	96.9%	98.9%	94.4%

Indicators	Ituri	North Kivu	Tshopo	Overall
% of travelers made aware	99.4%	96.9%	100%	98.6%
Number of alerts notified	9	2	0	11
Number of validated live alerts	6	0	0	6
Number of bodies recovered today	2	2	0	4
Number of problematic contacts intercepted today; % of	0	0	0	0
validated active alerts referred to CT/CTE	100.0% (6/6)	0.0%	0.0%	100.0% (6/6)
% of lifeless bodies swabed	100.0% (2/2)	100.0% (2/2)	N / A	100.0% (4/4)
Number of positive results of referred suspected cases	0	0	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator		Ituri	North Kivu Haut-Uele	Tshopo Together		
Number of beds		800	141	ND	ND	941
Patients in bed (Day -1)		576	167	ND	ND	743
Total admissions (24 hours)		102	49	ND	ND	151
Total admissions		678	216	ND	ND	894
Outings	Deceased	19	6	ND	ND	25
	No case	40	37	ND	ND	77
	Healed	13	2	ND	ND	15
	Escapees	2	0	ND	ND	2
	Transferred to the HGR	2	0	ND	ND	2
Total outflows (24 h)		76	45	ND	ND	121
Patients in isolation (end of day),		602	171	ND	ND	773
including confirmed cases		287	20	ND	ND	307
including suspects		315	151	ND	ND	466
Occupancy rate		75.3%	121.3%	ND	ND	82.1%

In total, 773 patients were in isolation, including 307 confirmed cases and 466 suspected cases, for an overall occupancy rate of 82.1 % (773 patients for 941 beds), with 894 admissions and 121 discharges (15 recovered, 25 deceased, 77 non-cases, and 2 who escaped). Ituri bears the brunt of the burden with 602 patients in isolation (287 confirmed, 315 suspected) out of 800 beds, representing an occupancy rate of 75.3%. In North Kivu, 171 patients are in isolation (20 confirmed, 151 suspected) for an occupancy rate of 121.3% (141 beds). Haut-Uele and Tshopo did not provide data at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	25 (96.2%)	2 (100%)
Confirmed cases are being monitored.	114 (89.1%)	1 (100%)
New suspected cases	10 (19.6%)	6 (100%)
Suspected cases are being monitored	125 (78.1%)	21 (100%)

SMSPS Key Indicators	Ituri	North Kivu
Laboratory results announced (including positive ones)	59 (33)	12 (2)
Children separated — nursery / community	0/0	0/0
CTE support staff	153	6
Frontline Personnel (PPL)	9	5
Members of households and communities	58	0
EDS supported (SMSPS)	5	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD

Health Zone	Confirmed cases	Cured. In hospital.		Death
Bunia	35	18	2	15
Rwampara	33	24	2	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	1	0	1	0
Total — lethality 30.1%	123	67	19	37

PPL = healthcare workers and frontline providers.

7 Pillar-Based Response Activities

COORDINATION Holding the coordination meeting with the various partners; monitoring field activities and consolidating daily reports in the affected provinces.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ŷ Ituri: Continued estimation of PCI input needs within the framework of continuity of essential services and care; Analysis of indicators drawn from DHIS2 (ongoing); Development and Budgeting of the CSSE pillar plan.
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LOGISTICS ÿ Ituri: 38 vehicles and 12 ambulances mobilized; deliveries of PCI/PECH inputs and laboratory equipment in several health zones, deployment of Starlink kits and support for the Rwampara CTE (100 operational beds). ÿ North Kivu: Preparation and delivery of inputs/MEG and PCI kits in priority health zones, with support for the mobility and supply of teams. ÿ Tshopo: Supervision of PoE/PoC, logistical support to PoE Bangboka/PoC ONATRA and continuation of water supply to PoC PK23 RN4.	PCI / WASH ÿ Ituri: 53 rings opened, 61 ESS monitored and accompanied, 8 ESS and 25 means of transport decontaminated; 182 PPL trained and 56/58 households decontaminated within 48 hours. A new PPL case was reported in Nizi. ÿ North Kivu: continuation of PCI monitoring in social and solidarity economy (SSE) and actions to reduce the risk of exposure of service providers. ÿ Tshopo : Continued triage, prevention and infection control activities in health facilities, with persistent need for IPC inputs.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 158 people sensitized door-to-door, 120 during educational talks, 31 during community dialogues and 1,335 during mass awareness campaigns; 77 community alerts reported); Closing of the training of 40 religious leaders of the AS RWAMPARA on the MVE and guided tour at the CTE RWAMPARA. ÿ North Kivu: Continued awareness-raising, community engagement and communication activities at crossing points and in priority areas; Closing of the briefing on Ebola Virus Disease (EVD) (CREC and SBC) for community leaders and journalists in the Masereka health zone. ÿ Haut-Uélé: Training of 70 RECOs (ZS Pawa 40 and Boma Mangbetu 30) and 25 ITs (10 in Pawa and 25 in Boma Mangbetu) on community-based surveillance with support from Village Reach.	HOLISTIC CARE (HCC) ÿ Ituri: 13 new recoveries, 602 patients in isolation, 102 new admissions and 76 discharges, including 19 deaths; 968 people received hot meals. ÿ North Kivu: 49 new admissions and 45 discharges (2 recovered, 6 deaths and 37 non-cases); 171 patients remained hospitalized. ÿ Tshopo: No new admissions; 4 non-cases discharged and no patients hospitalized. Experimental treatment continues for 2 contacts (Day 8) at the FEDI Health Center in Mangobo.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: Continuation of CAP collection in several health zones and preparatory work/clinical trials related to vaccination strategies and EBO-PEP.	Dignified and secure burials (DSM) ÿ Ituri: 65 death alerts reported, 50 EDS carried out and 40 bodies swabed; continued training of EDS teams in Nia-Nia and PK51. ÿ North Kivu: 33 death alerts reported; 33 bodies swabed, secured and 30 EDS carried out.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ Ituri: Continuation of activities on the ground.	

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify the CREC, involve religious and community leaders, and ensure the safety of the teams. increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 121.3% in North Kivu (bed capacity: 141) and 75.1% in Ituri (602 patients for 800 beds);	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Low contact tracing performance in Ituri (75.6%); North-Kivu 83.7%; Haut-Uele 64.8% and 90.9% Tshopo	Uninterrupted transmission chains Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risk (123 cases of PPL, including 37 death) Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Butembo and Mambasa)	Long confirmation delays, transmission not detected Accelerate the deployment of decentralized diagnostic capabilities
10	Financial of the resources shortcomings	Limitation of the implementation of the pillars Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Guided visit of religious leaders from AS RWAMPARA to CTE RWAMPARA on the sidelines of the closing of their training on Ebola Virus Disease in Ituri.



Closing of the briefing on the MVE (CREC and SBC) for community leaders and journalists from the Masereka health zone in North Kivu

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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With technical support from the World Health Organization (WHO) and
Africa CDC